OVERHEAD SHOULDER INSTABILITY AND TENDONITIS

Most Common Overhead Sport Shoulder Problems:

Bicipital Tendonitis
Shoulder Impingement

Most Common Reason for Overhead Shoulder Issues:

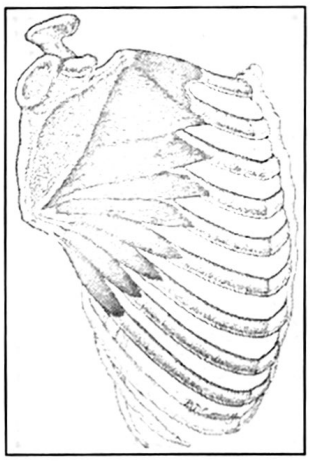
*The center of mass does not shift adequately over the 'stance' leg or contralateral leg, as the thorax abducts under the side that the shoulder is moving into flexion.

Most Common Mechanical Issues Contributing to Bicipital Tendonitis:

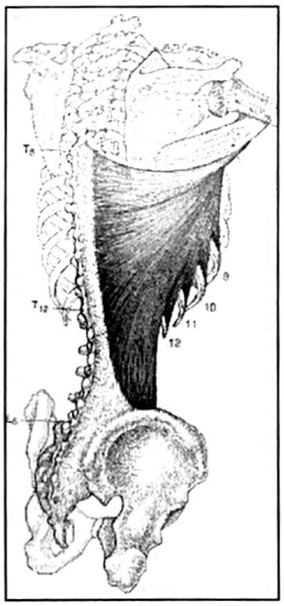
1. Decreased ability to rotate trunk or chest wall to the right.

2. Decreased ability to externally rotate ribs (turn ribs up in the front and retract rib cage, usually on the right secondary to forward shoulder position and weak serratus anterior muscle) during integrated left abdominal (IO/TA) co-contraction. See asterisked (*) activities on the following corresponding techniques page.

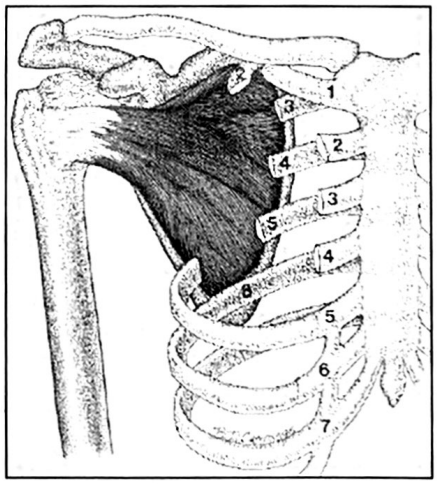
3. Overuse of the latissimus dorsi muscle for humeral (arm) internal rotation secondary to poor scapular (shoulder blade) position and therefore, subscapularis and serratus muscle function.



Serratus Anterior Muscle



Latissimus Dorsi Muscle



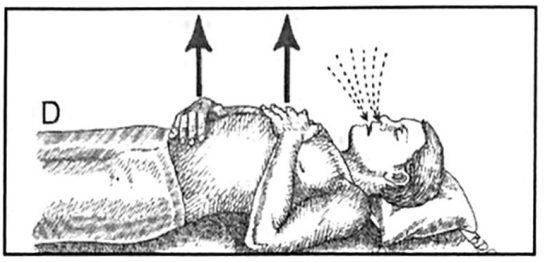
Subscapularis Muscle

Reference: Travell J and Simons D. Myofascial pain and dysfunction: the trigger point manual, upper half of body. Volume 1. Williams and Wilkins.

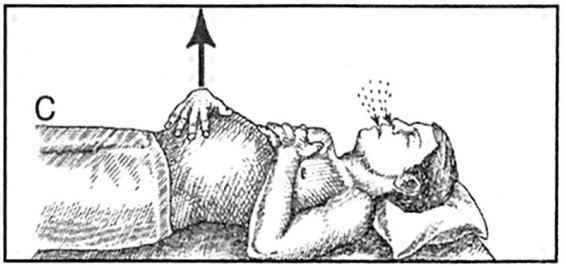
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Most Common Mechanical Issues Contributing to Shoulder Impingement:

1. Poor ability to expand rib cage with diaphragmatic breathing.

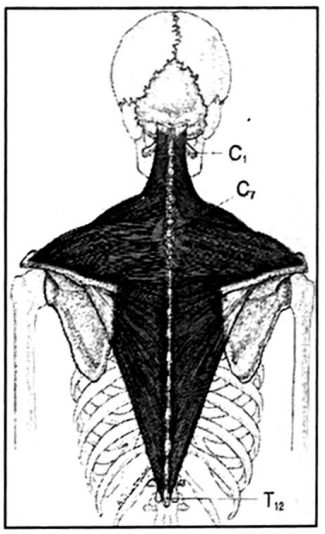


2. Poor ability to co-contract abdominals upon inhalation.

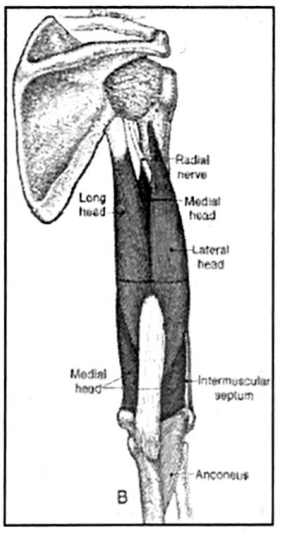


3. Over activity from the latissimus for indirect control of scapula through thoracic extension, accompanying scapular posterior tilting and scapular downward rotation.

4. Weak low trap and long head of the tricep (usually on the right) resulting in winging and anterior tilting of the shoulder blade.



Trapezius Muscles



Triceps Muscles

Reference: Travell J and Simons D. Myofascial pain and dysfunction: the trigger point manual, upper half of body. Volume 1. Williams and Wilkins.

Above lists also contribute to intra scapular weakness or weakness of the muscles between the shoulder blades.

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Postural Restoration Techniques for Right Bicipital Tendonitis with Left IO/TA's

1. Standing Supported Passive Left AF IR with Right Trunk Rotation
   (Integration – Standing #39)



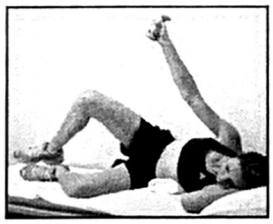
2. *Standing Wall Supported Resisted Serratus Punch
   (Integration – Standing #33)



3. Latissimus Hang with Low Trap
   (Right Lower Trapezius & Right Tricep – Standing #24)



4. *Left Sidelying Weighted Right HG Horizontal Abduction with Right FA ER
   (Right Lower Trapezius & Right Tricep – Sidelying #4)



5. Standing Resisted Bilateral Serratus Press Through
   (Integration – Standing #61)



6. *Left Sidelying Resisted Right HG IR
   (Integration – Sidelying #63)



7. Supine Resisted Right HG IR with Left HG ER
   (Right Lower Trapezius & Right Tricep – Supine #16)



8. Seated Resisted Right Abducted Diagonal Extension
   (Right Lower Trapezius & Right Tricep – Seated #4)



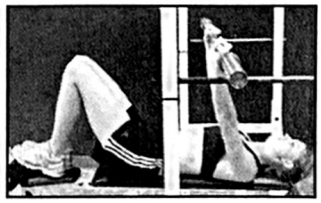
9. *Left Sidelying Resisted Right Serratus Punch with Right Trunk Rotation
   (Right Lower Trapezius & Right Tricep – Sidelying #7)



10. *Standing Resisted Right Diagonal Flexion in Right AIC
    (Integration – Standing #73)



11. Reversed "C" Push-Ups
    (Integration – Supine #35)



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Postural Restoration Techniques for Right Shoulder Impingement with Left IO/TA's

1. Supine Weighted Tricep Curls *(Right Lower Trapezius & Right Tricep – Supine #8)*



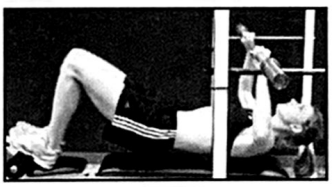
2. Supine Resisted Right Tricep Extension *(Right Lower Trapezius & Right Tricep – Supine #9)*



3. Standing Resisted Bilateral Tricep Pull Downs *(Integration - Standing #58)*



4. Bench Hooklying Thoracic Pull-Ups *(Integration – Supine #50)*



5. Seated Resisted Pull Downs *(Integration – Seated #20)*



6. Seated Resisted Alternating Tricep Pull Downs *(Integration – Seated #21)*



7. Seated Resisted Reciprocal Pull Downs *(Integration – Seated #22)*



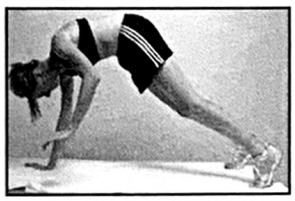
8. Long Seated Supported Press Down with Abdominals *(Integration – Long Seated #5)*



9. Paraspinal Release with Left Obliques *(Integration – Long Seated #8)*



10. All Four Belly Lift *(Integration – All Four #8)*



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